



STROKE

Effective: January 1, 2026
Replaces: January 1, 2025

1. Patient Care Goals

- 1.1. Detect neurological deficits
- 1.2. Determine eligibility for transport to a stroke center
- 1.3. Identify patients who have potentially sustained a stroke involving a large vessel occlusion (LVO)

2. BLS Treatment

- 2.1. Routine Medical Care – Adult (700-S04)
 - 2.1.1. **Oxygen** – titrate as appropriate
- 2.2. Determine **Blood Glucose Level** (BGL) (700-A03)
- 2.3. Complete **G.F.A.S.T.** stroke screening:

G	Gaze Abnormalities	(0-1)
F	Facial Asymmetry	(0-1)
A	Arm or leg Weakness/Drift	(0-1)
S	Speech Difficulties	(0-1)
T	Time Last Known Well	(no point)

- 2.4. Confirm patient has not had a seizure during the duration of stroke symptoms. If patient has had a seizure during the duration of stroke symptoms or is actively seizing, see (700-A02) and transport to appropriate Emergency Department.
- 2.5. Place patient in supine position with head elevated 30 degrees
- 2.6. Document time patient was last seen at baseline (Time Last Known Well) from a reliable historian
- 2.7. Determine if patient takes any anticoagulant medication, history of recent surgery or recent stroke
- 2.8. Provide advanced notification (**Policy 501**)
- 2.9. Do not delay transport for interventions and transport to the appropriate receiving facility, see section 2

3. Stroke Center Transport Determination

- 3.1. Patient will be considered a Stroke Alert Patient if the Time Last Known Well is within twenty-four (24) hours
- 3.2. If patient has four (4) points on the G.F.A.S.T stroke screening transport the patient to a Comprehensive Stroke Center (**Policy 602**).
 - 3.2.1. If transport time to closest Comprehensive Stroke Center is greater than forty-five (45) minutes, transport to the closest Primary Stroke Center.



- 3.3. If patient has three (3) or fewer points on the G.F.A.S.T stroke screening transport the patient to the closest Stroke Center (Comprehensive or Primary) (**Policy 602**).

4. ALS Treatment

- 4.1. **Vascular Access (IV)**, TKO
- 4.1.1. With no smaller than a 20-gauge catheter
 - 4.1.2. Antecubital (AC) access site is preferred
 - 4.1.3. Make no more than 1 attempt at the AC if first attempt fails
- 4.2. **Blood Glucose Level (BGL)**, if less than 60 mg/dL see Protocol [\(700-A03\)](#)
- 4.3. Obtain **12-Lead ECG** while enroute, if dysrhythmia or cardiac symptoms are present [\(700-A14\)](#)

5. Pertinent Assessment Findings

- 5.1. In patients with pre-stroke disability, stroke assessment should consider changes from patient's baseline and level of consciousness may need to be considered
- 5.2. Collect contact number for a reliable historian that can state the patient's "Time Last Known Well" and medical history. Inform them to expect contact by receiving hospital once patient arrives
- 5.3. If patient/family require interpreter services, provide details during advanced notification

6. Key Documentation Elements

- 6.1. "Time Last Known Well" must be specific. If patient was last known well prior to bedtime, this is the time to be documented, not the time they woke up with symptoms present
- 6.2. Use of GFAST stroke scale
- 6.3. Blood glucose results
- 6.4. Specific validated stroke scale used and findings
- 6.5. Time of notification to receiving hospital



7. Stroke Center Transport Determination Flow Chart

